

February 23rd, 2026

The Honorable Robert F. Kennedy Jr.
U.S. Department of Health and Human Services
200 Independence Ave SW
Washington, DC 20201

Re: Request for Information - Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care (RIN 0955-AA13)

Dear Secretary Kennedy:

On December 23rd, 2025, the Department of Health and Human Services (HHS) in partnership with Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP) and Office of the National Coordinator for Health Information Technology (ONC) (collectively, ASTP/ONC) released a Request for Information soliciting information regarding seek broad public comment on what HHS can do to accelerate the adoption and use of AI as part of clinical care. Point32Health welcomes the opportunity to provide insight into this critical matter.

Point32Health, the parent company of Harvard Pilgrim Health Care and Tufts Health Plan, is a leading not-for-profit health and well-being organization dedicated to delivering high-quality, affordable health care. Serving nearly 2 million members, our purpose is to guide and empower healthier lives.

As the Administration addresses emerging issues surrounding artificial intelligence, we would appreciate consideration of the below factors. We believe these factors are important to achieve the promise of AI and optimize its positive impact on the average American through increased efficiencies, lower costs, and improved quality of care.

i. Sustainability and Efficiencies

- *Savings Pass-Through:* If AI increases provider productivity and reduces administrative waste, those savings should be passed on to members, through their health plans, using proper coding and reduced rates. If AI is used to significantly reduce the necessity of human input, then it should be appropriately coded so that such savings flow through the health care system. In addition, federal coding should be re-evaluated as appropriate. For instance, the Resource Based Relative Value Scale (RBRVS) system currently assumes virtually all human execution.
- *Democratization of AI:* Smaller hospitals, providers, health plans and other elements of the health care system could benefit greatly from AI. However, cost prohibitive pricing from AI companies could squeeze out the very players that could benefit the most. Public policy should advance mechanisms through which smaller players can afford to access AI as well.

- *AI to AI:* As different players in the health care system and wider community begin utilizing AI, public policy should facilitate the efficient interaction between AI systems. For instance, if a hospital is using AI to provide claims information, it may be more efficient to have that AI interface with another AI rather than call a human customer service representative.

ii. Safety Standards

- *Human in the Loop:* A consistent requirement that humans must remain a part of the clinical decision-making process to ensure accountability.
- *Required Training:* The notion of “garbage in, garbage out” is critical in the AI space. Crafting appropriate prompts and assuring that critical patient data is communicated is critical to using AI in any clinical use case. Therefore, clinicians that will be utilizing AI as part of their practice should go through appropriate training. Medical schools haven’t prepared clinicians for this new technology. And while it is easy to develop any prompt for AI, it takes training and skill to develop effective prompts that will enable safe AI clinical support.
- *Health IT Safety Center:* Revisiting the idea of a national center focused specifically on the safety of AI-supported clinical decisions to prevent harm or death. Public Policy makers should ensure that AI vendors transparently disclose critical information that guided the development of their health care AI products. Experts should identify the critical elements, for instance – what populations were used to train the AI? Vendors should be required to publish ongoing statistics identified as critical by AI and health care experts – such as, what percentage of AI diagnoses are over-ruled by human clinicians? How often does AI fail to prompt clinicians for missing data critical to patient evaluation? Etc.
- *Broad Application of Rules:* Any AI system that influences a medical decision should be subject to the same high standards of transparency and safety, regardless of what that entity is called (e.g. non-medical devices, software as medical devices, digital therapeutic, etc.)

iii. Transparency and Traceability

- *Mandatory Tagging:* Any official documentation (claims, appeals, diagnoses) generated or assisted by AI must be explicitly labeled as such to distinguish it from human-generated work.
- *Chain of Custody for Logic:* There should be a requirement to maintain a record of the AI’s cognitive processes and the specific prompts used by clinicians to justify medical decisions.
- *Audit Requirements:* Healthcare entities using AI for clinical care should be subject to regular federal audits to address dangerous AI use or cost creep through upcoding. This should include:
 - o Prompt Transparency: Ability to access specific prompts used by clinicians to ensure they were accurate and unbiased.
 - o Overrule Rates: Tracking how often a human clinician disagreed with or overruled the AI’s recommendation as a metric for effectiveness.

- o Safety Reporting: Establishing a mechanism to report instances where AI-driven decision support resulted in incorrect treatment or patient harm.

iv. Regulatory Reform and Interoperability

- *Data Emancipation*: Public policy should prevent delivery systems (like hospital networks) from gatekeeping patient data. Providers and EMR vendors should be required to provide data for treatment payment and health care operations.
- *Updating Medicaid/Medicare Rules*: Existing regulations, such as network adequacy and staffing ratios (e.g., case manager-to-patient ratios), need to be updated to account for the efficiency gains provided by AI.
- *Interoperability*: Policymakers should ensure that as part of interoperability, delivery systems cannot charge high fees for data sharing. In addition, the interoperability rules should be evaluated to ensure that they will facilitate the use of AI with aggregated data to propel cost savings and efficiencies in the future. It would be a waste of opportunity – as well as significant funds – if AI applicability is not contemplated until AFTER interoperability is completely implemented.

v. AI Tools can Reduce Administrative Friction in the Healthcare System and Lower Medical Cost Trends for All Americans

- In 2020, the federal government established that if providers fail to adopt certified electronic health record (EHR) technology (CEHRT), Medicare could reduce their payment levels. Even though over 88% of Medicare providers have invested in electronic health records, provider administrative burden remains significant and problematic. The positive potential of EHR adoption has been undermined because remote access to EHRs usually isn't granted to payors at all, or only at an additional cost. In some cases, this is because the EHR vendors require providers to purchase additional licenses if payors are allowed to remotely access the EHR. As a result, providers must continue to use faxes and other outdated modes to communicate to health plans. Access obstacles to EMR data currently pose a significant barrier to private sector innovation in AI for health care and its adoption and use in clinical care. The integration of AI tools and Large Language Models to optimize the scaled aggregation of health care data should be applied across the healthcare delivery system to achieve real interoperability. The same standards for AI Data accessibility should apply to hospital networks, providers, EMR vendors and payors. This could be accomplished more rapidly if CMS were to update the CEHRT standards in coordination with adopting a uniform AI policy in health care as follows:
 - a) The EHR vendors would be required to allow payor organizations to remotely access a provider's EHR for treatment, payment and healthcare operations as defined by HIPAA, without the imposition of additional fees onto the provider client or the payor entity.
 - b) The provider licensing the EHR would be required to allow payor organizations to access a provider's EHR for treatment, payment and

healthcare operations as defined by HIPAA, without the imposition of additional fees onto the payor entity.

We appreciate that HHS has solicited comments on this emerging technology that is rapidly expanding in use. We look forward to collaborating with the federal government to advance meaningful use of AI in a manner that optimizes outcomes for all Americans while minimizing risks.

Please contact me at Christina.Nyquist@Point32Health.org if we can provide additional assistance.

Sincerely,



Christina Nyquist
Vice President, Federal Affairs